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OFFICIAL NDIS REPORT

NDIS Support Stage Progress Report

Report Date: 2026-06-20
Participant ID: 6XGFASFF

NDIS PARTICIPANT DETAILS

PARTICIPANT NAME
Johnathan Miller

NDIS PARTICIPANT ID
N123456789

PARTICIPANT EMAIL
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DATE OF BIRTH (DOB)
N/A

Official NDIS Audit & Progress Findings

PSYCHOSOCIAL ASSESSMENT REPORT

Practice Details:

Aleksandra Gorczynska-Kolodziej
ABN: 33257573623
ACA Registered Counsellor (Level II)
Email: office@horizonscc.com.au | Phone: 0401413634
Report Date: 2026-06-20

PARTICIPANT INFORMATION

Field	Details
Full Name	Marcus Williams
NDIS Number	789456123
Date of Birth	22/08/1985
Support Coordinator	Rebecca Chen (rebecca.chen@complexsupport.org.au / 0423 987 654)
Plan Management	Self-Managed
Assessment Period	05/06/2026 - 19/06/2026

1. EXECUTIVE SUMMARY (Support Coordinator Copy/Paste)

Purpose of Assessment:

Comprehensive psychosocial assessment to evaluate participant's mental health, emotional wellbeing, social connectedness, and coping capacity. This assessment informs NDIS planning for psychosocial and therapeutic supports under Capacity Building - Improved Daily Living.

Assessment Methods:

- 2x Clinical Interviews (4 hours total)
- Standardised psychosocial screening tools (PCL-5, DASS-21, K10, WHODAS 2.0, AUDIT, DAST-10, C-SSRS, LSNS-6, Brief-COPE, WEMWBS, ISI)
- Mental State Examination
- Collateral consultation with support network
- Narrative therapeutic consultation

Key Psychosocial Findings:

Participant presents with significant psychosocial distress characterised by severe PTSD symptoms, extremely severe depression and anxiety, hazardous alcohol use, and severe functional impairment. Participant reports complex trauma history including childhood abuse, homelessness, and justice system involvement. These factors substantially impact participant's capacity to engage in social, community, and daily living activities without support.

Total Recommended Psychosocial Support & NDIS Line Items:

- **Therapeutic Counselling:** 1 hour/week for ongoing basis

(NDIS Line Item: Assessment Recommendation Therapy or Training - Counsellor, Item 15_043_0128_1_3)

- **Psychosocial Recovery Coach / Support Worker:** 6 hours/week

(NDIS Line Item: Psychosocial Recovery Coach, Item 04_105_0125_6_1 OR Assistance with Social, Economic and Community Participation)

- **Review Period:** 3 months

2. PSYCHOSOCIAL ASSESSMENT TOOLS ADMINISTERED

Tool	Domain	Score	Interpretation
PCL-5	PTSD Symptoms	62/80	Severe PTSD Symptoms
GAD-7	Anxiety	19/21	Severe Anxiety
PHQ-9	Depression	20/27	Severe Depression
DASS-21	Depression / Anxiety / Stress	D:22 A:18 S:24	Extremely Severe Depression, Severe Anxiety, Extremely Severe Stress
K10	Psychological Distress	46/50	Very High Psychological Distress
WHODAS 2.0	Functional Impact	54/60	Severe Functional Impairment
LSNS-6	Social Network	2/18	Severely Limited Social Network
Brief-COPE	Coping Strategies	Predominantly Avoidant and Substance-Based	Maladaptive Coping Profile
WEMWBS	Mental Wellbeing	18/70	Very Low Mental Wellbeing
MSPSS	Perceived Support	8/49	Very Low Perceived Support
C-SSRS	Suicide Risk	Category 3	Moderate-High Risk - Ideation with Some Intent
AUDIT	Alcohol Use	24/40	Hazardous Alcohol Use
DAST-10	Drug Use	7/10	Moderate Drug Use Problems
ISI	Sleep	22/28	Severe Insomnia

3. PSYCHOSOCIAL FUNCTIONING ASSESSMENT

DOMAIN 1: MENTAL HEALTH & EMOTIONAL WELLBEING

STATUS: SEVERE AND COMPLEX IMPAIRMENT

Clinical Observations:

- Participant reports chronic emotional dysregulation, hypervigilance, and intrusive trauma memories
- PCL-5 score of 62 indicates Severe PTSD symptoms
- DASS-21 indicates Extremely Severe Depression (22), Severe Anxiety (18), and Extremely Severe Stress (24)
- K10 score of 46 indicates Very High Psychological Distress
- Mental State Examination reveals: Hypervigilant affect, flattened emotional expression, psychomotor retardation, evidence of dissociative episodes during interview, guarded and mistrustful interpersonal style, poor concentration
- WEMWBS score of 18 indicates Very Low Mental Wellbeing
- ISI score of 22 indicates Severe Insomnia affecting daily functioning

Psychosocial Impact Statement:

Due to severe PTSD symptoms, depression, and anxiety, participant experiences significant emotional distress that impacts their capacity to engage in all daily activities, maintain housing stability, and manage basic self-care. Emotional dysregulation and trauma responses are compounded by substance use as a coping mechanism.

Recommendations:

- Therapeutic counselling focusing on phase-oriented trauma-informed approach, cognitive behavioral therapy, and emotional regulation
- Psychoeducation on trauma responses and coping strategies
- Regular monitoring of psychological distress levels and suicide risk

DOMAIN 2: SOCIAL CONNECTION & RELATIONSHIPS

STATUS: SEVERE ISOLATION AND DYSFUNCTION

Clinical Observations:

- LSNS-6 score of 2 indicates Severely Limited Social Network
- MSPSS score of 8 indicates Very Low Perceived Social Support
- Participant reports: "I don't trust anyone. People have always let me down or hurt me. I'm better off alone."
- WHODAS interpersonal relationships domain: Severe difficulty (score 5/5)
- Participant has no close friends or family contact. Estranged from family of origin (history of abuse). Minimal contact with community services beyond mandatory appointments.
- Interpersonal style characterised by mistrust, hypervigilance, and difficulty forming attachments

Psychosocial Impact Statement:

Participant experiences profound social isolation and interpersonal dysfunction that compounds mental health difficulties. Limited social support reduces resilience and increases vulnerability during periods of distress. Mistrust and trauma history create barriers to forming therapeutic and supportive relationships.

Recommendations:

- Psychosocial support worker / Recovery Coach to facilitate community connection
- Graduated social engagement with therapeutic support
- Goal: Develop at least 1 meaningful social connection within 6 months

DOMAIN 3: COPING & RESILIENCE

STATUS: SEVERE DEFICITS - MALADAPTIVE COPING

Clinical Observations:

- Brief-COPE profile indicates Predominantly Avoidant and Substance-Based coping strategies
- Participant reports: "Drinking and using is the only way I can cope. When I'm sober, the memories and feelings are too much."
- Current coping strategies: Alcohol use (daily, hazardous levels), cannabis use (4-5x/week), methamphetamine use (weekly), social withdrawal, emotional numbing
- No adaptive coping strategies identified
- Substance use functions as primary coping mechanism for trauma symptoms and emotional pain

Psychosocial Impact Statement:

Participant has severely limited repertoire of adaptive coping strategies, relying primarily on substance use and avoidance. This creates a cycle of trauma symptoms ' substance use ' negative consequences ' increased trauma symptoms. Without alternative coping strategies, recovery is unlikely.

Recommendations:

- Skills-based counselling to develop adaptive coping strategies
- Psychoeducation on emotion regulation techniques and harm reduction
- Practice opportunities with psychosocial support worker
- Referral to drug and alcohol service for specialised support

*DOMAIN 4: DAILY PSYCHOSOCIAL FUNCTIONING***STATUS: SEVERE IMPAIRMENT****Clinical Observations:**

- WHODAS overall score of 54 indicates Severe Functional Impairment
- Participant requires support with all activities of daily living, community access, self-care, household management, financial management, health management
- Impact on: self-care (poor hygiene, inconsistent nutrition), household management (currently homeless), community access (limited to survival activities), social participation (minimal), employment (unemployed 5 years)
- Sleep disruption (ISI score: 22) affecting daily functioning
- Participant currently experiencing chronic homelessness (3 years), sleeping rough or in crisis accommodation
- No current income beyond Newstart Allowance
- Poor physical health (dental issues, chronic pain, untreated medical conditions)

Psychosocial Impact Statement:

Psychosocial difficulties substantially impact participant's capacity to engage in any daily activities. Homelessness, substance use, and trauma symptoms create a cycle of survival that prevents engagement in recovery activities. Without intensive support, participant is at risk of continued homelessness, deteriorating health, and justice system involvement.

Recommendations:

- Intensive psychosocial support worker assistance with all daily living activities
- Immediate priority: stable housing (supported accommodation or transitional housing)
- Therapeutic work on basic self-care and routine building
- Coordination with housing services, health services, and income support
- Harm reduction approach to substance use

*DOMAIN 5: TRAUMA & SAFETY***STATUS: SEVERE AND ONGOING RISK****Clinical Observations:**

- PCL-5 score of 62 indicates Severe PTSD symptoms
- Participant reports history of:
 - Childhood trauma: Physical abuse, emotional abuse, neglect (ages 5-15)
 - Adolescent trauma: Sexual assault (age 16), homelessness (age 17-19)
 - Adult trauma: Domestic violence (multiple relationships), assault during homelessness (3 incidents)
- Current safety:
 - Housing: Currently sleeping rough - UNSAFE environment
 - Substance use: Hazardous alcohol use, moderate drug use - health and safety risk
- Suicide risk: Moderate-High (C-SSRS Category 3) - ideation with some intent
- Self-harm: Historical cutting, currently abstinent 6 months
- Vulnerability: High risk of exploitation, assault, and further trauma due to homelessness and substance use
- Participant reports frequent nightmares, flashbacks, hypervigilance, and emotional numbing
- Dissociative episodes observed during assessment

Psychosocial Impact Statement:

Participant experiences ongoing trauma due to current unsafe living situation (homelessness) and historical complex trauma. The combination of severe PTSD symptoms, substance use, and homelessness creates a high-risk situation requiring immediate intervention. Without stable housing and intensive trauma-informed support, participant is at risk of further trauma, health deterioration, and death.

Recommendations:

- IMMEDIATE PRIORITY: Secure safe, stable housing (supported accommodation or transitional housing)
- Phase-oriented trauma-informed counselling (stabilisation first, trauma processing later)
- Safety planning and crisis intervention protocols
- Intensive case management and coordination between all services
- Regular monitoring of suicide risk and mental health stability
- All providers to be informed of trauma-informed care principles

4. MENTAL HEALTH RISK ASSESSMENT

Risk Domain	Level	Details
Suicide/Self-Harm	MODERATE-HIGH	C-SSRS: Category 3 - Ideation with some intent but no specific plan. Participant reports frequent thoughts of "not wanting to be here anymore" and "wishing I could just disappear." No current plan or intent, but access to means. Protective factors: None identified - no social support, no reasons for living identified. Safety plan: Prepared but limited effectiveness due to lack of social support.
Self-Neglect	SEVERE	Requires intensive monitoring and support. Participant demonstrates severe self-neglect: poor hygiene, inconsistent nutrition, untreated medical conditions, sleeping in unsafe environments.
Vulnerability	SEVERE	Homelessness, substance use, and trauma history create extreme vulnerability. High risk of exploitation, assault, and further trauma. Participant reports being assaulted 3 times while sleeping rough in past year.
Hospitalisation Risk	MODERATE	Risk of psychiatric hospitalisation if mental health deteriorates or suicide risk increases. Currently no acute indicators, but high baseline risk.

Safety Plan: Prepared but limited effectiveness due to lack of social support and unstable housing

Crisis Contacts Provided: Lifeline (13 11 14), Suicide Call Back Service (1300 659 467), local mental health triage, emergency services (000)

GP Informed: Yes - participant's GP has been notified of assessment findings and recommendations

5. THERAPEUTIC RECOMMENDATIONS

SHORT-TERM GOALS (0-3 months)

Goal #	NDIS Goal Alignment	Therapeutic Intervention	Frequency	Duration	Success Indicator
1	"I want to stop using so much"	Harm reduction approach to substance use, development of alternative coping strategies	Weekly	Ongoing	Reduction in alcohol and drug use by 25%
2	"I want to feel safer"	Safety planning, stabilisation techniques,	Weekly	12 sessions	Participant demonstrates use of

		grounding skills for trauma symptoms			grounding techniques during distress
3	"I want to sleep better"	Sleep hygiene intervention, nightmare management techniques	Integrated	Ongoing	ISI reduction from 22 to <15

LONG-TERM GOALS (3-12 months)

Goal #	Target Outcome	Timeline
4	Stable housing and reduced substance use	3-6 months
5	Sustainable mental wellbeing (PCL-5 <40, DASS-21 <15)	9-12 months
6	Development of supportive social network	12 months
7	Engagement in meaningful activity (employment, volunteering)	12 months

6. PSYCHOSOCIAL SUPPORT STRATEGIES FOR SUPPORT WORKERS

TOTAL RECOMMENDED HOURS: 6 hours/week

Breakdown:

Activity	Hours/Week	Description
Housing Support	1.5 hrs	Assistance with housing applications, accompanying to housing appointments, support in transitional accommodation
Daily Living Support	1.5 hrs	Prompting and support with self-care, nutrition, hygiene, medical appointments, financial management
Community Connection	1 hr	Accompanying to community activities, peer support groups, drug and alcohol services
Emotional Support	1 hr	Active listening, compassionate presence, crisis support, reinforcement of coping strategies
Skill Practice	1 hr	Practicing grounding techniques, distress tolerance, emotion regulation in real-world settings
TOTAL	6 hrs/week	

PSYCHOSOCIAL SUPPORT APPROACH

DO:

- [x] Use person-centred, trauma-informed approach
- [x] Practice harm reduction approach to substance use (non-judgmental, realistic goals)
- [x] Provide consistent, predictable support (builds trust with trauma survivor)
- [x] Offer choice and control in all interactions (counteracts powerlessness from trauma)
- [x] Use clear, transparent communication (no surprises, explain everything)
- [x] Respect boundaries and autonomy (don't push, follow participant's pace)
- [x] Validate emotional experiences without minimising
- [x] Maintain professional boundaries while being warm and compassionate
- [x] Report changes in mental health, substance use, or safety concerns to counsellor immediately
- [x] Coordinate closely with all other service providers

DON'T:

- [x] Use coercive or controlling approaches (retraumatising)
- [x] Judge or shame substance use (creates disengagement)

- [x] Make promises you can't keep (breaks trust)
- [x] Take challenging behaviours personally (they're trauma responses)
- [x] Push trauma processing before stabilisation is achieved
- [x] Ignore safety concerns or minimise risk
- [x] Work in isolation - coordinate with team

EMOTIONAL SUPPORT STRATEGIES

When participant is in crisis (suicidal ideation, severe dissociation):

1. Ensure immediate physical safety
2. Use calm, grounding presence
3. Assess level of risk using C-SSRS
4. Activate safety plan
5. Contact crisis services if needed (000, mental health triage)
6. Stay with participant until stable
7. Debrief with counsellor and team

When participant is using substances:

1. Non-judgmental approach
2. Assess immediate safety (overdose risk, unsafe behaviours)
3. Provide harm reduction information
4. Offer support to access drug and alcohol services
5. Reinforce motivation for change when present
6. Report to counsellor and team

When participant is experiencing trauma symptoms (flashbacks, dissociation):

1. Grounding techniques (5-4-3-2-1 sensory awareness, orientation to present)
2. Calm, reassuring presence
3. Offer choice and control
4. Reduce environmental stimulation
5. Stay with participant until grounded
6. Report to counsellor

7. COORDINATION REQUIREMENTS

Actions Required from Support Coordinator:

- [] **IMMEDIATE (Within 1 week):**
- URGENT: Secure stable housing (supported accommodation or transitional housing) - participant is currently sleeping rough and at high risk
- Implement intensive psychosocial support worker hours (6 hrs/week) under Assistance with Social, Economic and Community Participation OR Psychosocial Recovery Coach (Item 04_105_0125_6_1)
- Source trauma-informed support workers with experience in complex trauma, homelessness, and substance use
- Source counsellor for therapeutic sessions (ongoing) under Assessment Recommendation Therapy or Training - Counsellor (Item 15_043_0128_1_3)
- Review NDIS plan for adequate Capacity Building funding (Improved Daily Living)
- Coordinate with drug and alcohol services for integrated support
- [] **WITHIN 1 MONTH:**
- Coordinate initial meeting between counsellor, support workers, housing provider, and drug and alcohol service

- Ensure all providers understand trauma-informed approach and high-risk presentation
 - Confirm service agreements signed
 - Arrange psychiatric assessment for medication management
 - Coordinate physical health assessment and treatment
 - Develop integrated care plan with all providers
- [] **ONGOING (Weekly initially, then fortnightly):**
 - Monitor participant engagement with services and mental health stability
 - Weekly team meetings between all providers (counsellor, support workers, housing, drug and alcohol, GP)
 - Regular risk assessment and safety planning
 - Adjust support hours based on need
- [] **BEFORE PLAN REVIEW (3 months prior):**
 - Request updated psychosocial progress report from counsellor
 - Compile evidence for NDIA plan review
 - Demonstrate progress toward goals and ongoing need for intensive support

8. REVIEW TIMELINE

Review Type	Timeline	Assessment Tools	Success Indicators
Clinical Review	1 month	PCL-5, DASS-21, C-SSRS	Stabilisation, reduced acute risk
Progress Report	3 months	Full reassessment	Housing stability, reduced substance use, improved engagement
Plan Review	6 months	Comprehensive review	Evidence for ongoing intensive support

Report Validity: This assessment is valid for 6 months for NDIS planning purposes (given complexity and risk level, more frequent review recommended).

9. SCOPE OF PRACTICE STATEMENT CRITICAL

This assessment was conducted by a registered counsellor within their professional scope of practice. It focuses strictly on psychosocial and mental health functioning.

This report DOES NOT constitute:

- IA Functional Behavioural Assessment (FBA)
- IA Behaviour Support Plan (BSP)
- IA clinical psychological diagnosis (e.g., specific DSM-5/ICD-11 diagnosis)
- IA neuropsychological assessment
- IA psychiatric assessment
- IA drug and alcohol treatment plan (specialised assessment required)
- IA an assessment of physical functioning, mobility needs, or home modification requirements. *(To be assessed by an Occupational Therapist if required).*

For the following, separate referral is recommended:

- Behavioural support needs ' Positive Behaviour Support Practitioner (PBS)
- Formal psychological diagnosis (PTSD, substance use disorder) ' Registered Psychologist (AHPRA) or Psychiatrist
- Psychiatric assessment/medication ' Psychiatrist
- Specialised drug and alcohol treatment ' Drug and Alcohol Service
- Housing support ' Housing and Homelessness Services
- Occupational therapy needs ' Occupational Therapist

Collaborative Practice:

This counsellor is available to collaborate with psychiatrists, psychologists, drug and alcohol services, housing providers, and other allied health professionals to provide integrated, trauma-informed support for the participant.

10. CLINICIAN DETAILS

- **Name:** Aleksandra Gorczynska-Kolodziej
- **Professional Registration:** ACA Member (Registration No: 86625)
- **NDIS Worker Screening Check:** [x] Cleared
- **Professional Indemnity Insurance:** [x] Current
- **Blue Card:** [x] Current
- **Professional Description:** Registered clinical counsellor specializing in cognitive behavioral therapy (CBT), psychodynamic approach, trauma-informed care, mental health care, family and system therapies, and comprehensive psychosocial assessments. Experience working with complex trauma, substance use, homelessness, and high-risk presentations.

Contact:

- **Email:** office@horizonscc.com.au
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Signature: Aleksandra Gorczynska-Kolodziej

Date: 2026-06-20